

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Pneumovax 23 or Prevenar 13 to vaccinate against Pneumococcal Disease

Patient Group Direction, version 006, issued 11 September 2026. Supersedes Version 005, 11 September 2026.

Summary of the governing guidance for pneumococcal vaccination: Green Book chapter 25 (Pneumococcal), edition of 29 July 2026, the JCVI advice of June 2023 that PPV23 or PCV20 may be used in the adult and at-risk programmes, the JCVI advice of June 2024 on people experiencing homelessness, and the Prevenar 20, Prevenar 13 and Pneumovax 23 SmPCs. NICE CKS 'Immunizations - pneumococcal' summarises the same programme.

Note: this summary describes the national programme as set out in the July 2026 Green Book chapter, which moved adults at 65 and the clinical risk groups from PPV23 to Prevenar 20 (PCV20) in early 2026 and includes the infant schedule. The products authorised under this PGD are Prevenar 13 and Pneumovax 23 only, for individuals aged 2 years and over. Prevenar 20 and the infant schedule are outside this PGD: where national guidance indicates PCV20 for the individual and it is not held, refer to the GP.

Overview

- Pneumococcal disease is caused by *Streptococcus pneumoniae*, leading to infections such as **pneumonia**, **septicaemia**, and **meningitis**.
- Vaccination is aimed at **preventing invasive pneumococcal disease (IPD)**, especially in high-risk groups.

Pneumococcal Vaccines Available in the UK (Green Book Table 1)

1. **Pneumococcal conjugate vaccines (PCV): Prevenar 13 (PCV13), Vaxneuvance (PCV15), Prevenar 20 (PCV20) and Capvaxive (PCV21):**
 - **Prevenar 13 is the routine infant vaccine. Prevenar 20 has been used in the adult (65 years) and at-risk programmes since early 2026 and replaces PPV23 as national stock. Vaxneuvance (from 6 weeks) and Capvaxive (from 2 years, approved by the MHRA in 2026) are licensed but not in the national programme.**
2. **Pneumococcal polysaccharide vaccine (PPV23, Pneumovax 23):**
 - **Protects against 23 serotypes. Children under 2 years respond poorly and there is no evidence of effectiveness below that age.**
 - Recommended for risk groups since 1992 and for everyone aged 65 and over since 2003. PPV23 is no longer available to order through the national programme; locally held stocks are used up first, then PCV20.

Routine Childhood Schedule (UK)

- **PCV13 (Prevenar 13) is given as:**
 - **A single priming dose at 16 weeks of age (moved from 12 weeks on 1 July 2025 so that the second MenB dose can be given at 12 weeks)**
 - **A booster dose at 1 year of age (on or after the first birthday), with the other vaccines due at that age**

Doses of any PCV given before 12 weeks of age are discounted. An unimmunised or partially immunised child aged 1 to under 2 years has a single dose of PCV13; routine PCV is not offered after the second birthday. The infant schedule is outside this PGD.

Clinical Risk Groups (Green Book Table 2)

Offer a single dose of PPV23 or PCV20 to individuals aged 2 years and over with:

- **Asplenia or splenic dysfunction (including homozygous sickle cell disease, hereditary spherocytosis, thalassaemia major and coeliac disease with splenic dysfunction); chronic respiratory disease (COPD, bronchiectasis, cystic fibrosis, interstitial lung fibrosis, pneumoconiosis, bronchopulmonary dysplasia, and neurological or neuromuscular disease that compromises respiratory function; asthma only where it needs continuous or frequently repeated systemic steroids); chronic heart disease; chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant); chronic liver disease (cirrhosis, biliary atresia, chronic hepatitis)**
- **Diabetes mellitus requiring insulin or anti-diabetic medication (not diabetes that is diet controlled only)**
- **Immunosuppression due to disease or treatment, including chemotherapy, bone marrow transplant, asplenia or splenic dysfunction, complement disorder, HIV infection at all stages, multiple myeloma, genetic disorders affecting the immune**

system (IRAK-4, NEMO), and systemic steroids for more than a month at a dose equivalent to prednisolone 20mg or more a day (or 1mg/kg a day in a child under 20kg)

- Cochlear implants (immunisation must not delay the implantation)
- Cerebrospinal fluid leaks following trauma or major skull surgery (not CSF shunts)
- Occupational risk from frequent or continuous exposure to metal fumes, such as welders (PPV23, PCV20 or PCV21, one dose if none given before); and people experiencing homelessness (rough sleepers and those using homeless hostels or night shelters, JCVI June 2024)

Children under 2 years in a clinical risk group receive additional PCV doses on the Green Book Table 3 schedule (PCV20 for asplenia, splenic dysfunction, complement disorder or severe immunocompromise); this is outside this PGD.

Older Adults

- All adults are offered a single dose of PPV23 or PCV20 at 65 years of age. An individual who has already received PPV23 or PCV20 because they are in a clinical risk group does not need another dose at 65, whatever the interval. Revaccination every 5 years with PPV23 or PCV20 is recommended only for asplenia, splenic dysfunction (including sickle cell disease) and chronic kidney disease; it is not recommended for any other risk or age group. Antibody testing before vaccination is not required.

Vaccination Schedule by Group (Green Book chapter 25, July 2026, simplified)

Group	Vaccine	Notes
Infants	PCV13 at 16 weeks and a booster at 1 year (on or after the first birthday)	Routine; outside this PGD
Adults aged 65 and over	PPV23 or PCV20, single dose	Even if healthy. No dose at 65 where PPV23 or PCV20 was already given for a risk group
Clinical risk groups aged 2 years and over (other than the next row)	One PPV23 or one PCV20	No further PCV13 is needed from 2 years of age, whatever the earlier PCV history (Green Book Table 3). Repeat every 5 years only for asplenia, splenic dysfunction or chronic kidney disease
Asplenia, splenic dysfunction,	Asplenia, splenic dysfunction or	Requires specialist input.

Group	Vaccine	Notes
complement disorder or severe immunocompromise, aged 2 years and over	complement disorder: one PPV23 or PCV20. Severe immunocompromise (bone marrow transplant, acute or chronic leukaemia, multiple myeloma, genetic immune disorders): one dose of PCV20 whatever the previous PCV history, then PPV23 or PCV20 at least 4 weeks later; bone marrow transplant recipients have two PCV20 doses at least 4 weeks apart	Prevenar 20 is not authorised under this PGD: refer. Where Prevenar 13 is given under this PGD and PPV23 is also indicated, this PGD keeps the 8 week interval stated in the Prevenar 13 arm, which is longer than the Green Book minimum of 4 weeks

Contraindications

- Confirmed anaphylactic reaction to a previous dose of a pneumococcal vaccine, or to any component or residue from the manufacturing process. When in doubt seek specialist advice rather than withholding vaccine.
- Acute illness: postpone until fully recovered. Minor illness without fever or systemic upset is not a reason to postpone.
- Pregnancy and breastfeeding: pneumococcal vaccines may be given in pregnancy when protection is needed without delay; there is no evidence of risk from inactivated vaccines in pregnancy or breastfeeding.
- Timing around splenectomy or immunosuppressive treatment: vaccinate ideally 4 to 6 weeks, and at least 2 weeks, before elective splenectomy, chemotherapy or radiotherapy; where that is not possible, vaccinate at least 2 weeks after splenectomy or at least 3 months after completing chemotherapy or radiotherapy (leukaemia: 6 months after chemotherapy; bone marrow transplant: from 9 to 12 months after transplant). Treatment is never delayed for vaccination, and vaccination is not delayed where that would mean it never happens.

Side Effects

- PPV23: mild soreness and induration at the injection site for 1 to 3 days, less commonly a low-grade fever; reactions are commoner in people with high existing antibody levels. PCV13 (children): injection site reactions, fever, irritability, decreased appetite, sleep changes. PCV20 (adults): injection site pain, muscle pain, fatigue, headache, joint pain.
- Rare: allergic reaction including anaphylaxis.

Co-Administration

- Pneumococcal vaccines can be given at the same time as any other vaccine (including influenza and COVID-19) at a separate site, preferably a different limb, or at least 2.5cm apart in the same limb. Record the site of each vaccine.
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Documentation

- Record the vaccine name and brand, batch number, expiry date, site, date and route.
- Report adverse reactions via the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Pneumovax 23

for vaccination against

Pneumococcal Disease

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Pneumovax 23 is indicated for active immunisation against pneumococcal disease caused by the 23 serotypes included in the vaccine for individuals aged 2 years and over at increased risk, in accordance with national guidance.
Inclusion criteria	- Individuals aged 2 years and over eligible for pneumococcal vaccination under national guidance. - Informed consent obtained from the individual or guardian. - No contraindications to pneumococcal vaccination.
Exclusion criteria	- Known hypersensitivity to Pneumovax 23 or any of its components. - Previous severe allergic reaction to any pneumococcal vaccine. - PPV23 or PCV20 received within the last 5 years. Revaccination with Pneumovax 23 is authorised only for asplenia, splenic dysfunction or chronic kidney disease, and only where at least 5 years have elapsed since the previous dose; it is not recommended for any other group. - Acute illness with fever (postpone until recovered).
Cautions	- Use with caution in individuals with bleeding disorders. - Record batch number, expiry date and site of administration. - Monitor for anaphylaxis for 15 minutes after administration. - Inform patient of possible side effects and when to seek help.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Pneumovax 23
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular or subcutaneous injection.
Dose and frequency	Single 0.5 mL dose. Revaccination every 5 years ONLY for asplenia, splenic dysfunction or chronic kidney disease (Green Book chapter 25); not recommended for any other group, and never within 3 years of a previous dose.
Quantity to be administered and/or supplied	0.5 mL per dose.
Maximum or minimum treatment period	One lifetime dose in most cases. Repeat only under specific clinical recommendations.
Adverse effects	Injection site pain, redness, swelling, fever, fatigue, headache. Rare: allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Attend for any further dose of the course on the date given. Seek medical advice for a severe or persistent injection site reaction, a fever that does not settle within 48 hours, or any sign of an allergic reaction. Call 999 for difficulty breathing, swelling of the face or throat, or collapse. Where a Pneumovax 23 dose is due after Prevenar 13, attend for it on the date given (at least 8 weeks later).

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 25 Pneumococcal; JCVI advice on the pneumococcal programme, current; Summaries of Product Characteristics for the vaccine administered, current version
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
11/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Prevenar 13



for vaccination against

Pneumococcal Disease

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prevenar 13 is indicated for active immunisation against pneumococcal disease caused by the 13 serotypes included in the vaccine, for infants, children, and adults in accordance with the national immunisation programme and clinical risk group recommendations.
Inclusion criteria	- Individuals aged 2 years and over who are eligible for pneumococcal vaccination under national guidance and in whom a conjugate vaccine is indicated. - Infant primary immunisation (under 2 years) is NOT given under this PGD. - Informed consent obtained from the individual or guardian. - No contraindications to Prevenar 13.
Exclusion criteria	- Known hypersensitivity to the active substance or any component of the vaccine, including hypersensitivity to diphtheria toxoid (CRM197 carrier protein). - Severe allergic reaction to a previous pneumococcal conjugate vaccine. - Aged under 2 years. - Acute febrile illness (postpone vaccination until recovery).
Cautions	- Observe the patient for 15 minutes post-vaccination. - Use with caution in patients with bleeding disorders. - Record batch number, expiry date and site of administration in the patient record. - Advise on expected side effects and when to seek medical advice.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Prevenar 13
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection.
Dose and frequency	Single 0.5 mL dose for individuals aged 2 years and over who have not previously received a pneumococcal conjugate vaccine. Where PPV23 is also indicated, give it at least 8 weeks after the conjugate vaccine.
Quantity to be administered and/or supplied	0.5 mL per dose.
Maximum or minimum treatment period	A single dose under this PGD. Infant primary and booster doses are not given under this PGD.

Adverse effects	Irritability, drowsiness, loss of appetite, fever, injection site redness/swelling. Rare: hypersensitivity reactions. Refer to SmPC.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Follow-up advice to be given to patient or carer	Attend for any further dose of the course on the date given. Seek medical advice for a severe or persistent injection site reaction, a fever that does not settle within 48 hours, or any sign of an allergic reaction. Call 999 for difficulty breathing, swelling of the face or throat, or collapse. Where a Pneumovax 23 dose is due after Prevenar 13, attend for it on the date given (at least 8 weeks later).

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 ● NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources ● UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 25 Pneumococcal; JCVI advice on the pneumococcal programme, current; Summaries of Product Characteristics for the vaccine administered, current version ● British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) NICE
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● Summary of Product Characteristics

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group	Job title & name of	Signature	Date



signatory

organisation

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025



Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.



Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 10: the guidance summary was rewritten against Green Book chapter 25 of 29 July 2026 (Prevenar 20 in the adult and at-risk programmes from early 2026 with PPV23 stock used up first, Capvaxive licensed but not in the programme, infant PCV13 at 16 weeks and 1 year, the Table 2 clinical risk groups including occupational metal fume exposure and people experiencing homelessness, no further PCV13 from 2 years of age, PCV20 for severe immunocompromise, 5-yearly revaccination only for asplenia, splenic dysfunction and chronic kidney disease, contraindications, timing around splenectomy and immunosuppressive treatment, pregnancy, side effects by product and co-administration); the Pneumovax 23 and Prevenar 13 arms are unchanged and the summary states that Prevenar 20 is not authorised under this PGD and that the Prevenar 13 arm keeps its 8 week interval before PPV23.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary now carries a note that it describes the national programme (Prevenar 20 from July 2026, and the infant schedule) while this PGD authorises Prevenar 13 and Pneumovax 23 only, from 2 years of age.

The Pneumovax 23 exclusion for a dose within the last 5 years no longer carries the circular 'unless 5 years have elapsed' exception; revaccination is stated as the dose row has it, for asplenia, splenic dysfunction or chronic kidney disease only and only after 5 years.

Both cautions rows now ask for the batch number, expiry date and site; previously one arm asked for batch and expiry and the other for batch and site.

The Prevenar 13 maximum treatment period row now says a single dose under this PGD instead of 'including primary and booster doses', which the inclusion criteria exclude.

The follow-up advice rows, which carried the treatment template about 3 to 4 weeks, now give vaccination follow-up advice.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name Green Book chapter 25, JCVI and the SmPCs as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.

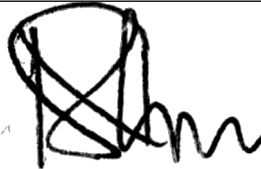
Pneumovax 23: exclusion added for PPV23 or PCV20 received within the last 5 years. The SmPC says revaccination within 3 years is not recommended and the Green Book restricts 5-yearly revaccination to asplenia, splenic dysfunction and chronic kidney disease. The vague 'may be considered' line is replaced with that rule.

Prevenar 13: hypersensitivity to diphtheria toxoid (the CRM197 carrier) added as an SmPC contraindication. Inclusion restricted to 2 years and over; infant primary immunisation is not given under this PGD and 'according to the national immunisation schedule' is replaced by a stated dose.

The guidance summary has not yet been rewritten against the July 2026 Green Book chapter (Prevenar 20, infant dose at 16 weeks). That is recorded in the review register for the next version.

Signed on behalf of Get Real Health

Version v006, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.